

When is it safe for Dad to drive?

Driving windows after Ativan, Dayvigo, doxepin+trazodone, and quetiapine · 2026-05-22 (quetiapine section added 2026-05-24)

For Dad, Mom, Leah,
David,
Debbi, family team

THE SINGLE MOST IMPORTANT THING ON THIS PAGE

How Dad feels is not a reliable signal of whether he's safe to drive. Pomara 2015 showed that elderly patients on low-dose lorazepam — even 0.25 mg — had measurable psychomotor impairment *without subjectively feeling impaired*. "I feel fine" is not the test. The clock is the test, and on doses that span the day, family-observed steadiness and reaction time are the test.

QUICK REFERENCE — MINIMUM NO-DRIVE WINDOWS AFTER A DOSE

Scenario	Minimum no-drive	Safer window
Dayvigo (lemborexant) 5 mg at bedtime, no Ativan	~8 hours	Until 10 AM
Ativan 0.5 mg, alone	12 hours	24 hours
Ativan 1 mg, alone (the morning rescue dose)	24 hours	48 hours
Ativan + Dayvigo same evening	12 hours	Family check through afternoon
Ativan 0.5 mg AM after doxepin 6 mg + trazodone 50 mg HS (old stack)	12 hours	16 hours
Ativan 1 mg AM after doxepin 6 mg + trazodone 50 mg HS (old stack)	16 hours	No driving day-of
Any of the above + Depakote (if started)	Double the window	Skip driving day-of
Quetiapine 25–50 mg HS only — single dose at 9 PM	16 hr (~1 PM)	4 PM or next AM
Quetiapine 25 mg HS + 25 mg midnight (split-dose, current pattern)	16 hr (~4 PM)	No drive day-of
Quetiapine ANY dose during the first 5–7 days of bridge	No driving — titration window per FDA label	
Quetiapine + any lorazepam same calendar day	No driving that day	
Through 5/26 (residual nordoxepin overlap)	No driving regardless of quetiapine timing	

QUICK HEURISTIC FOR THE QUETIAPINE BRIDGE — SINGLE-DOSE 9 PM SCENARIO

If quetiapine is taken as a **single 25–50 mg dose at 9 PM** (no midnight redose), **16-hour rule = ~1 PM minimum driving. BUT five conditions must all be met:** (1) past Day 7 of the regimen (initial titration window); (2) past 5/26 (residual nordoxepin from recent doxepin course); (3) no lorazepam taken that day; (4) family pre-drive check passed (Section below); (5) short familiar daylight route only — no highway, no dusk/night driving, no long trips, no passengers depending on him.

Scenario timelines

 **No drive** — drug at peak / clinically meaningful levels

 **Caution** — family check required

 **Cleared** — drug below impairment threshold

Scenario 1 — Dayvigo at bedtime only (the routine sleep stack)

~8 HR MINIMUM

Lemborexant 5 mg HS taken at 10 PM. No Ativan.



The published OE analyses covered Ativan-driving directly but didn't address Dayvigo-alone driving explicitly — the windows below are a conservative interpretation of the FDA Dayvigo label (warns of potential next-day impairment), lemborexant's 17-hour half-life, and Dad's specific risk profile. The Dayvigo SUNRISE driving studies in working-age adults found minimal impairment the morning after a 5 mg dose. For Dad, given prodromal Lewy-spectrum markers and gait abnormality, **prefer not to drive before 10 AM** the morning after a bedtime dose. Family-observed steadiness check before driving is the floor.

Scenario 2 — Ativan 0.5 mg, used during the day

12 HR MINIMUM, 24 HR SAFER

Lorazepam 0.5 mg taken at noon. No other CNS-depressant additions today.



A 0.5 mg dose produces measurable psychomotor impairment for at least 12 hours in elderly patients per Pomara 2015. **Don't drive on the same calendar day as a 0.5 mg dose.** The next morning, if Dad needs to drive, a family member should check steadiness, gait, and reaction time first.

Scenario 3 — Ativan 1 mg (the morning-rescue size dose)

24 HR MINIMUM, 48 HR SAFER

Lorazepam 1 mg taken at 7 AM, e.g., to abort an acute anxiety episode like the one on 5/21 morning.

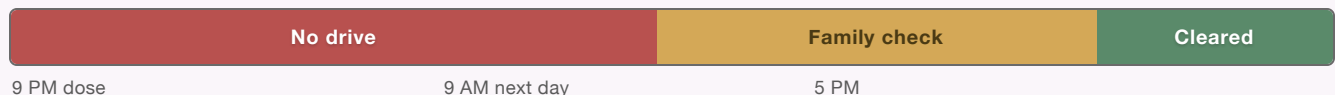


The 1 mg dose used on 5/21 morning is twice the half-life-clearance burden of the 0.5 mg scenario. The FDA injectable label specifies 24–48 hours no-driving after a 1 mg dose; Seppälä 1976 found measurable impairment at 12+ hours from a 2.5 mg dose in young healthy volunteers. For Dad, **plan for two no-drive days** after any 1 mg dose. If Dad needs to drive on day 2, family check first.

Scenario 4 — Ativan in the evening + Dayvigo at bedtime

12 HR MIN; CAUTION THROUGH AFTERNOON

Lorazepam 0.5 mg at 9 PM (pre-bed anxiety rescue) + lemborexant 5 mg at 10 PM. Additive CNS depression overnight.



OE published guidance: **12 hours minimum after the Ativan dose**, with the caveat that additive CNS depression and lemborexant's 17–19 hour half-life mean residual effects overlap through the next day. The lemborexant FDA label specifically warns of increased daytime impairment when combined with benzodiazepines, especially in elderly patients. **Don't drive in the morning**; if Dad must drive in the afternoon, family steadiness check first. Default to late-afternoon or skip-the-day if function isn't time-pressed.

Scenario 5 — Doxepin 6 mg + trazodone 50 mg HS (the old stack, if Dad is still on it)

8–12 HR MIN; FAMILY CHECK

Doxepin 6 mg HS + trazodone 50 mg HS taken at 10 PM. No Ativan today. No lemborexant.



The complication on this stack is that **doxepin's active metabolite (nordoxepin) has a 31-hour half-life** and accumulates over consecutive nights to steady state by ~5 days. Trazodone clears mostly by morning (elderly half-life ~8 hours, ~35% remaining at 12 hours) — but the doxepin/nordoxepin component lingers and the residual sedation grows night by night during the first week. **Family-observed steadiness check before any morning driving is the operative tool**, especially after the first 3–5 nights of dosing. The FDA-labeled elderly starting dose for doxepin is 3 mg, not 6 mg — worth raising with the prescribing team given the residual-sedation question (see callout below).

Scenario 6 — Ativan AM rescue on top of the old stack from **12–16 HR MIN; LIKELY NO DRIVE DAY-OF** the night before

Doxepin 6 mg + trazodone 50 mg HS taken last night (10 PM). Then Ativan 0.5–1 mg taken AM today as PRN rescue.

No drive		Family check	Cleared
8 AM Ativan dose	midnight	8 AM next day	

The morning Ativan layers GABA-A sedation on top of residual nordoxepin (31-hr half-life), residual trazodone (~35% remaining at 12 hours), and any accumulated steady-state from prior nights. Three distinct sedative mechanisms overlap. **For 0.5 mg Ativan: 12 hr minimum, 16 hr safer. For 1 mg Ativan: 16 hr minimum, 24 hr safer — effectively no driving that day**, and probably no driving the next morning either. Plus the dominant safety concern on this combination is fall risk, not just driving (see callout below).

DOXEPIN + TRAZODONE CONTEXT — IMPORTANT IF ON THE OLD STACK

Nordoxepin (doxepin's active metabolite) has a 31-hour half-life. This is the longest-acting component in any sleep stack containing doxepin. It accumulates over consecutive nights, reaching steady state at ~5 days. After a week of dosing, residual next-morning sedation is meaningfully higher than after the first night. This is why the family-steadiness check matters more — and gets harder to pass — the longer the regimen runs.

The FDA-labeled doxepin starting dose in elderly (≥65) is 3 mg, not 6 mg. Dad is on the maximum labeled dose for any age group and double the recommended elderly starting dose. If the prescribing team isn't already aware, raising this with them — and asking whether 3 mg would give comparable sleep with less residual sedation — is worth doing.

Trazodone is not a low-risk fall agent in elderly. Bronskill 2018 found trazodone's fall-injury rate (5.7% at 90 days in nursing homes) was statistically equivalent to benzodiazepines (6.0%; HR 0.94). Rivasi 2025 found trazodone users ≥75 had 58.3% syncope/falls vs. 21.2% in non-users ($p=0.001$). Combined with Dad's baseline BP 112/58 and sinus bradycardia (HR 49–53), trazodone is a meaningful fall-risk contributor on its own.

Antidepressant + benzodiazepine combinations independently raise fall risk. Wang 2025: moderate-dose antidepressant + low-dose benzodiazepine = HR 1.71 for injurious falls; higher doses = HR 1.96. Dad on duloxetine + Ativan PRN, with doxepin/trazodone added at night, is in the higher-risk band of this finding.

IF DEPAKOTE (DIVALPROEX) GETS STARTED — READ THIS CAREFULLY

Valproate **doubles Ativan's effective blood level** by slowing its clearance by ~40%. The FDA label mandates reducing the Ativan dose by 50% when valproate is on board, because a 0.5 mg dose becomes pharmacokinetically equivalent to a 1 mg dose.

If Depakote ends up on board, double every window above. A 0.5 mg Ativan dose now requires the 24-hour window. A 1 mg Ativan dose requires 48 hours minimum. The Dayvigo + Ativan combination becomes a clear "no driving day-of" call.

Quetiapine driving scenarios (added 2026-05-24)

Quetiapine 25–50 mg HS is now the working sleep agent for the ECT bridge. The published pharmacokinetic and driving-impairment evidence (Fauska 2026, Karsten 2017, Solhaug 2023, Carr 2023) supports a **16-hour minimum no-drive window from the last quetiapine dose** in elderly patients, with conditions. The three scenarios below cover the most likely use cases.

Scenario 7 — Quetiapine 25–50 mg as a single dose at 9 PM, no midnight redose

16 HR MIN (~1 PM)

Quetiapine 25 or 50 mg HS taken at 9 PM. No midnight redose. No lorazepam today. Past Day 7 of the bridge regimen. Past 5/26 (nordoxepin washout complete).



Pharmacokinetics in elderly: quetiapine IR has half-life ~6 hours in general population, but **oral clearance is reduced 30–50% in patients ≥65, extending effective half-life to ~8–10 hours** (FDA label; Solhaug 2023 TDM study n=8,118). At 16 hours post-dose (1 PM after 9 PM dose), residual plasma concentration is ~10–15% of peak — approximately 2 elderly half-lives, the threshold at which driving simulator performance returns toward baseline. **1 PM minimum, 4 PM safer.** Family pre-drive check still required. Restrict to short, familiar, daylight routes.

Scenario 8 — Quetiapine 25 mg HS + 25 mg midnight (the current split-dose pattern)

16 HR MIN (~4 PM)

Quetiapine 25 mg at ~10 PM bedtime + another 25 mg at midnight when waking. Total ~50 mg overnight in split-dose schedule. No lorazepam today. Past Day 7 of bridge.

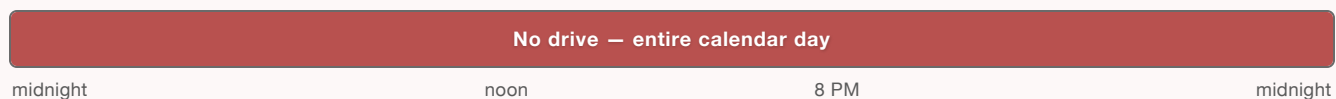


The midnight dose resets the pharmacokinetic clock. Instead of measuring from the 9 PM bedtime dose, measure from the midnight dose. 16 hours after midnight = 4 PM. **Mid-to-late afternoon driving (after 4 PM) is pharmacokinetically plausible** if all conditions are met. **Morning and early-afternoon driving is not safe** on the split-dose pattern. If a third dose at 3 AM is ever taken, no driving the entire following day.

Scenario 9 — Quetiapine + any lorazepam on the same calendar day

NO DRIVING DAY-OF

Any quetiapine dose the night before + any lorazepam dose (0.5 mg or 1 mg) the next day.



Dassanayake 2011 meta-analysis: benzodiazepines are associated with 60–80% increased crash risk. Combined with residual quetiapine the morning after, the additive CNS-depressant burden is substantial. **On any day Dad takes lorazepam, driving is off the table — full stop.** This applies regardless of timing (AM rescue dose or scheduled).

THROUGH 5/26: NO DRIVING AT ALL (NORDOXEPIN OVERLAP)

Doxepin's active metabolite nordoxepin has a **31-hour half-life** and accumulates over the recent ~17-night doxepin course. **Through approximately 5/26**, meaningful nordoxepin levels persist alongside quetiapine — triple CNS-depressant overlap (quetiapine + nordoxepin + duloxetine). This represents an unstudied combination for driving safety in elderly. **Default: no driving through 5/26 regardless of quetiapine dose timing.**

Also: **first 5–7 days on quetiapine = no driving.** The FDA label notes somnolence is most pronounced during initial dose titration. This is the standard antipsychotic-initiation precaution and applies fully here.

Net practical implication for Adam's heuristic question (5/24): the "9 PM dose → 1 PM driving" heuristic is pharmacokinetically correct but only applies **after both conditions clear (after Day 7 of the regimen AND after 5/26)**. Until then, no driving.

LEWY-SPECTRUM PROFILE LOWERS THE THRESHOLD FURTHER

Thompson 2018 meta-analysis: Parkinson disease patients show significant driving impairment on both simulator and on-road testing compared to healthy controls. Crizzle 2012: driving may be impaired even in early-stage PD. Dad's documented gait abnormality (R26.9), balance problem (R26.89), peripheral neuropathy (G62.9), and chronic RBD represent a constellation that, in the context of possible prodromal Lewy-spectrum disease, **substantially lowers the threshold at which medication-induced sedation becomes dangerous for driving**. El-Saifi 2016 found significantly greater falls and injury in patients with parkinsonism on quetiapine. The cumulative risk is multiplicative, not additive.

Practical implication: formal occupational therapy driving evaluation (CDRS — Certified Driver Rehabilitation Specialist) would be the strongest-evidence option for resolving the driving question through the ECT bridge. Worth raising with U-M ECT team and PCP Dr. Straka.

Family check before driving — the steadiness test

IF DAD IS IN ANY "AMBER / FAMILY CHECK" WINDOW ABOVE, RUN THROUGH THESE BEFORE HE DRIVES

1. **Stand from a seated position without using arms for push-off.** Slowed? Off-balance? Don't drive yet.
2. **Walk 20 feet in a straight line, turn, walk back.** Watch for hesitation, drift, or shortened steps relative to his baseline.
3. **Stand still with feet together, eyes closed, for 10 seconds.** Sway? Step-out for balance? Don't drive.
4. **Brief reaction-time check:** drop a pen, ask Dad to catch it. Or stand at arm's length and ask him to tap your hand as soon as you raise it. Slowed reaction relative to baseline? Don't drive.
5. **Quick conversation:** ask Dad to tell you the date, his next appointment, and what he had for breakfast. Word-finding hesitation, confusion, or unusual flatness of voice? Don't drive.
6. **Ask Dad how he feels.** If he says "fine" but any of items 1–5 looked off, the assessment is "don't drive," not "fine." Pomara 2015: subjective feel is unreliable.

TWO OBJECTIVE TESTS ADDED FOR QUETIAPINE-BRIDGE DRIVING DECISIONS (VALIDATED FOR ELDERLY DRIVING PREDICTION)

1. **Trail Making Test A (TMT-A)** — **printable, timed**. Connect numbers 1–25 in order. **Threshold: >54 seconds → no driving** (Papandonatos 2015 + Kokkinakis 2021). Free printable sheets available online; takes ~1 minute. The most evidence-supported predictor of driving impairment in older adults on sedating medications.
2. **Timed Up and Go (TUG)**. Stand from chair, walk 3 meters (10 feet), turn, walk back, sit. **Threshold: >12 seconds → no driving** (Kokkinakis 2021). Combine with TMT-A: **3+ positive tests on the combined battery = 3.46-fold increased likelihood of unsafe driving**.

These objective tests are particularly useful during the quetiapine bridge because (a) Dad's gait/balance baseline is already documented as abnormal, so subjective family observation is harder to calibrate, and (b) the OE evidence found TMT-A specifically validated for predicting driving impairment in elderly on sedating medications. Establish baseline times now (when Dad is at his best) for comparison during the bridge.

Other points worth knowing

- **Alcohol changes everything**. Even one drink within the no-drive window or the family-check window above resets the timer to "no drive day-of." Lemborexant + alcohol is specifically warned against on the FDA label.
- **Time of dose matters**. A morning Ativan dose extends the no-drive window through the entire day; a bedtime dose mostly burns through overnight. If Ativan is needed, prefer evening dosing when possible — but don't game timing to enable driving.
- **Cumulative use changes the calculus**. The windows above are for occasional PRN use. If Ativan use becomes near-daily, the windows extend further and family steadiness checks become the operative tool day-to-day.
- **The bigger picture**. If Dad's medication regimen is producing frequent no-drive days that limit his independence, that's information worth bringing to the U-M conversation. The goal isn't to maximize the windows — it's to get to a stable regimen where the driving question isn't a daily problem.

Quetiapine driving evidence — ranked by quality (added 2026-05-24)

★★★★★ = robust. ★★★★☆ = strong. ★★★☆☆ = moderate. ★★☆☆☆ = sparse.

★★★★★

Quetiapine Fumarate FDA Label. Updated 2025-09-11.

U.S. Food and Drug Administration.

Finding: Patients should be cautioned about activities requiring mental alertness, such as operating a motor vehicle, until they are "reasonably certain that quetiapine therapy does not affect them adversely." Plasma clearance reduced 30–50% in elderly. **Applies:** The label's subjective "reasonably certain" standard is unreliable in elderly on sedating medications (Pomara 2015); the time-based 16-hour rule operationalizes the label.



American Geriatrics Society 2023 Updated Beers Criteria.

J Am Geriatr Soc 2023;71(7):2052-2081. doi:10.1111/jgs.18372.

Finding: Antipsychotics (including quetiapine) listed as potentially inappropriate in older adults. Lowest dose for shortest time with ongoing risk-benefit assessment. CNS-active polypharmacy flagged. **Applies:** Reinforces the no-driving-on-Ativan-days rule and the formal CDRS referral recommendation as the conservative path.



Fauska C, Bastiampillai T, Rawson G et al. Quetiapine Modestly Improves Sleep but Impairs Next Day Performance in OSA: A Randomized Controlled Trial.

Ann Am Thorac Soc 2026. doi:10.1093/annalsats/aaog092.

Finding: 50 mg quetiapine IR at bedtime vs placebo, crossover RCT. **Next-morning driving simulator performance significantly impaired** (steering deviation 96 ± 53 vs 72 ± 38 cm, $p < 0.05$). The only published controlled trial of low-dose quetiapine and driving. **Applies:** Demonstrates that even at 50 mg — the lowest dose studied — quetiapine produces objectively measurable next-morning driving impairment. Not in elderly specifically, but the elderly half-life is longer (clearance reduced 30–50%) so impairment likely extends further.



Solhaug V, Tveito M, Waade RB et al. Impact of Age, Sex and CYP Genotype on Quetiapine and N-Desalkylquetiapine Serum Concentrations.

Br J Clin Pharmacol 2023;89(12):3503-3511.

Finding: Real-world TDM study ($n = 8,118$). Concentration-to-dose ratios increase by 28–29% in patients > 70 years. Oral clearance reduced 30–50% in elderly. **Applies:** Pharmacokinetic basis for the extended elderly half-life (~ 8 – 10 hr vs ~ 6 hr general population) and therefore the 16-hour minimum no-drive window.



Carr DB, Beyene K, Doherty J et al. Medication and Road Test Performance Among Cognitively Healthy Older Adults.

JAMA Netw Open 2023;6(9):e2335651.

Finding: Prospective cohort in adults ≥ 65 . **SSRI/SNRI use associated with $2.68 \times$ risk of marginal/fail on road test;** sedative/hypnotic use $2.72 \times$; independent and additive. **Applies:** Quantifies the additive driving-impairment burden from duloxetine + quetiapine. Direct evidence for elderly population.



Dassanayake T, Michie P, Carter G, Jones A. Effects of Benzodiazepines, Antidepressants and Opioids on Driving: A Systematic Review and Meta-Analysis.

Drug Safety 2011;34(2):125-56.

Finding: Benzodiazepines associated with 60–80% increased crash risk. Effects strongest in elderly and in combination with other CNS depressants. **Applies:** Anchors the "no driving on any day lorazepam is taken" rule when quetiapine is on board.



Thompson T, Poulter D, Miles C et al. Driving Impairment and Crash Risk in Parkinson Disease: A Systematic Review and Meta-Analysis.

Neurology 2018;91(10):e906-e916.

Finding: PD patients show significant driving impairment on both simulator and on-road testing vs. healthy controls. Effects influenced by disease stage and medication use. **Applies:** Lowers the threshold for safe driving in Dad's possible-prodromal-Lewy-spectrum profile (gait abnormality + balance + neuropathy + RBD).



Karsten J, Hagenauw LA, Kamphuis J, Lancel M. Low Doses of Mirtazapine or Quetiapine for Transient Insomnia: A Randomised, Double-Blind, Cross-Over, Placebo-Controlled Trial.

J Psychopharmacol 2017;31(3):327-337.

Finding: 50 mg quetiapine in healthy young men: next-day daytime sleepiness and lessened sustained attention vs placebo. **Applies:** Reinforces the next-day-impairment finding in non-elderly. Elderly burden expected to exceed this.



Kokkinakis I, Vaucher P, Cardoso I, Favrat B. Assessment of Cognitive Screening Tests as Predictors of Driving Cessation: A Prospective Cohort Study.

PLoS One 2021;16(8):e0256527.

Finding: TMT-A >54 seconds, TMT-B >150 seconds, abnormal clock-drawing, TUG >12 seconds all associated with increased driving cessation. Combined battery (3+ positive) = 3.46× increased likelihood. **Applies:** Anchors the TMT-A + TUG objective pre-drive check protocol added to the family steadiness test.



Papandonatos GD, Ott BR, Davis JD, Barco PP, Carr DB. Clinical Utility of the Trail-Making Test as a Predictor of Driving Performance in Older Adults.

J Am Geriatr Soc 2015;63(11):2358-64.

Finding: TMT-A and TMT-B are valid predictors of driving performance in older adults. TMT-A completion time >54 sec is a concerning threshold. **Applies:** Strongest specific validation for the TMT-A threshold used in the pre-drive check protocol.



El-Saifi N, Moyle W, Jones C, Tuffaha H. Quetiapine Safety in Older Adults: A Systematic Literature Review.

J Clin Pharm Ther 2016;41(1):7-18.

Finding: Quetiapine somnolence 25–39%, dizziness 15–27% in elderly. **Significantly greater falls and injury specifically in patients with parkinsonism.** **Applies:** Direct relevance to Dad's possible-prodromal-Lewy-spectrum profile.



American Psychiatric Association. Resource Document on the Role of Psychiatrists in Assessing Driving Ability (2023). Carr DB, Ott BR. The Older Adult Driver With Cognitive Impairment. JAMA 2010;303(16):1632-41.

Combined guidance.

Finding: Family informant reports with specific questioning about driving behaviors are more informative than patient self-assessment. Formal driving evaluation (CDRS) when concerns exist. **Applies:** Anchors the formal evaluation recommendation as the strongest-evidence option for resolving the driving question during the bridge.

Windows above reflect OpenEvidence-summarized published guidance (FDA labels for oral and injectable lorazepam, doxepin, trazodone, lemborexant, quetiapine; Pomara 2015 elderly psychomotor study; Seppälä 1976 driving impairment study; Greenblatt 1987 trazodone PK in elderly; Bayer 1983 trazodone in elderly; Rivasi 2025 trazodone-syncope; Seppala 2018 psychotropic fall-risk meta-analysis; Bronskill 2018 trazodone-vs-benzodiazepine falls; Wang 2025 antidepressant-benzodiazepine fall HR; Johnell 2017 psychotropic polypharmacy and fall hospitalization) interpreted for Dad's specific profile. **Quetiapine driving section (added 2026-05-24)** draws from Fauska 2026 RCT in OSA (50 mg quetiapine → next-morning simulator impairment); Karsten 2017 RCT (50 mg quetiapine next-day sleepiness in young men); Solhaug 2023 TDM study (n=8,118; concentration-to-dose ratios 28–29% higher in patients >70); Carr 2023 JAMA Netw Open prospective cohort (SSRI/SNRI use aHR 2.68, sedative/hypnotic 2.72 for road test failure in ≥65); Dassanayake 2011 benzodiazepine driving meta-analysis (60–80% increased crash risk); Thompson 2018 PD driving meta-analysis; El-Saifi 2016 quetiapine in elderly systematic review (parkinsonism-specific falls/injury); Kokkinakis 2021 + Papandonatos 2015 (TMT-A >54 sec threshold validation in elderly driving); AGS Beers 2023; AHA Tisdale 2020 drug-induced arrhythmias; APA 2023 Resource Document on driving assessment; Carr + Ott 2010 JAMA on family informant assessment. Recommendations interpreted for Dad's specific profile (age 76, sinus bradycardia, prodromal Lewy-spectrum markers including gait abnormality and balance problems, peripheral neuropathy, concurrent duloxetine, current quetiapine bridge, residual nordoxepin through ~5/26, ECT planned 2–4 weeks). U-M and Dr. Maxner may give updated guidance once they take over psychiatric care; the strongest-evidence option for resolving the driving question during the bridge is a formal occupational therapy CDRS (Certified Driver Rehabilitation Specialist) evaluation.